

RIVERSIDE MULTISPECIALTY HOSPITAL

Department of Endocrinology

Annual Comprehensive Diabetes Review

Patient Name:	John Doe	Medical Record #:	MRN-2026-001247
Age / Sex:	55 years / Male	Date of Visit:	2027-07-15
Consultant:	Dr. Sarah Smith, MD (Endocrinology)		

REASON FOR VISIT

Annual comprehensive review for long-term management of Type 2 Diabetes Mellitus and Essential Hypertension.

INTERVAL HISTORY

Mr. John Doe presents for his annual diabetes assessment approximately eighteen months after his initial diagnosis. Since his previous visit, he reports continued adherence to all prescribed medications and has successfully maintained the healthy lifestyle changes introduced over the past year. He continues walking approximately 45 minutes daily, prepares most meals at home, and routinely monitors his blood glucose. He reports feeling energetic and has returned to his normal work schedule without difficulty.

He explicitly denies excessive thirst, frequent urination, blurred vision, chest pain, shortness of breath, episodes of hypoglycemia, foot numbness, or foot ulcers. No hospital admissions or emergency department visits have occurred since the previous review.

CURRENT MEDICATIONS

Medication	Dose	Status
Metformin	500 mg orally twice daily	Continue
Insulin Glargine	8 units nightly	Continue
Amlodipine	5 mg orally once daily	Continue

PHYSICAL EXAMINATION & VITALS

General Exam Summary: Well-appearing patient in no acute distress.

Parameter	Current Visit	Previous Visit
Blood Pressure	124/78 mmHg	126/80 mmHg
Heart Rate	70 bpm	72 bpm
Respiratory Rate	16 / min	16 / min
Temperature	98.0 °F	98.1 °F
Weight	83 kg (-1 kg)	84 kg

Parameter	Current Visit	Previous Visit
Height	173 cm	173 cm
BMI	27.7 kg/m²	28.1 kg/m²

LABORATORY RESULTS

Test Description	Current Result	Previous Result
HbA1c	6.6%	6.9%
Fasting Plasma Glucose	98 mg/dL	104 mg/dL
Random Blood Glucose	132 mg/dL	138 mg/dL
Serum Creatinine	0.9 mg/dL	0.9 mg/dL
eGFR	>90 mL/min/1.73m²	>90 mL/min/1.73m²
Urine Albumin-Creatinine Ratio	Normal	Not Previously Tested
Total Cholesterol	170 mg/dL	178 mg/dL
LDL Cholesterol	98 mg/dL	108 mg/dL

ANNUAL DIABETES COMPLICATION SCREENING

Diabetic Foot Examination NORMAL Skin completely intact. No signs of ulceration. Peripheral pulses are fully palpable. Protective sensation remains fully preserved under 10 g monofilament testing.	Ophthalmology Review NORMAL Dilated retinal examination completed. There is zero clinical evidence pointing to diabetic retinopathy.
Renal Assessment NORMAL Renal function remains beautifully preserved. There is no biological evidence of diabetic nephropathy.	Cardiovascular Risk NORMAL Blood pressure values are cleanly controlled. No evidence of macrovascular or secondary cardiovascular complications.

ASSESSMENT

- Type 2 Diabetes Mellitus:** Well controlled. Displays significant, sustained improvement in glycemic control compared with initial clinic presentation. Supported by excellent long-term therapeutic adherence.
- Essential Hypertension:** Well controlled. Clean management achieved via existing daily Amlodipine regimen.
- Weight Status:** Successful long-term maintenance of weight reduction (Current BMI: 27.7).
- Complication Review:** Zero clinical markers or evidence of diabetic microvascular or macrovascular complications.

TREATMENT PLAN

- Prescription Continuation:** Continue Metformin 500 mg twice daily, Insulin Glargine 8 units nightly, and Amlodipine 5 mg daily.
- Lifestyle Maintenance:** Maintain current structured exercise routines and adhere tightly to the diabetic meal plan.

- **Monitoring Protocols:** Continue regular home blood glucose checks. Repeat HbA1c screening intervals scheduled every 6 months.
- **Preventative Health:** Schedule an annual diabetic eye examination, annual comprehensive foot review, and continue annual renal function monitoring.

PHYSICIAN SUMMARY & PATIENT EDUCATION

Since his initial diagnosis in January 2026, Mr. John Doe has demonstrated substantial improvement in the management of his Type 2 Diabetes Mellitus. Although temporary deterioration in glycemic control previously required escalation to basal insulin therapy, the patient responded exceptionally well through consistent medication adherence, structured diabetes education, dietary modification, and regular physical activity.

Current clinical findings indicate stable diabetes control, well-controlled hypertension, preserved renal function, and no evidence of diabetes-related complications. The patient was actively encouraged to continue these lifestyle measures for metabolic stability. Long-term prognosis is highly favorable provided current management strategies are maintained.

FOLLOW-UP

Return to clinic in 6 months for a routine review. Continue the structured annual comprehensive diabetes assessment cycle.

Electronically Signed By:

Dr. Sarah Smith, MD
Consultant Endocrinologist
Riverside Multispecialty Hospital